

SMMT: HARMONi-2 clears the Keytruda bar in China; the Western question is still open

Post-WCLC review · NEUTRAL VIEW · Price ~\$17.85 (approx., 20 Sep 2026) [28] · Market cap ~\$14bn **Date:** 24 September 2026 · **Analyst:** Chi Leong Andy Fu

Bottom line

WCLC delivered the best week of data ivonescimab has had. Akeso's HARMONi-2 became the first randomized Phase III trial to show an overall survival (OS) win against pembrolizumab (Keytruda) [2]. Summit's global HARMONi trial now has a survival confidence interval that no longer crosses 1 [3].

Neither result answers the question Summit's value depends on: **does the benefit hold in Western patients, with chemotherapy on board, against pembrolizumab plus chemotherapy?** Only Summit's HARMONi-3 can answer that, and its squamous cohort already missed an interim look in April [12].

After a ~44% move in 30 days [28], much of the good news is in the price. We stay Neutral into the HARMONi-3 squamous final PFS. The thresholds that would move us are in Section 4.

Company background

What Summit is. Summit Therapeutics (Nasdaq: SMMT) was founded in 2003 and is headquartered in Miami [1]. It began as an antibiotic developer. In July 2026, it sold its last legacy asset, ridinilazole, to Biossil [4], leaving a **single-asset oncology company** with no approved product and no revenue.

Chairman and co-CEO Robert Duggan controls the company with a stake of nearly 75% [11]. Duggan previously ran Pharmacyclics and sold it to AbbVie for \$21bn in 2015 [11].

The asset. Ivonescimab (SMT112 in Summit's territories, AK112 elsewhere) was engineered by China's Akeso [1]. It is a bispecific antibody that blocks two targets in one molecule:

- **PD-1**, the immune checkpoint Keytruda targets, which releases the brakes on the immune response.

- **VEGF**, which tumours use to grow blood vessels and to suppress immune cells.

Summit describes the four-arm (tetravalent) design as binding PD-1 more strongly where VEGF is present. That is meant to concentrate activity in tumour tissue rather than healthy tissue [1]. The investment thesis is that this could replace Keytruda-based regimens, starting in lung cancer.

The Akeso deal. Summit in-licensed ivonescimab in December 2022 for \$500m upfront, in a deal worth up to \$5bn [8][10]. Akeso receives low-double-digit royalties on sales [9]. Summit's territories are the US, Canada, Europe and Japan, expanded in June 2024 to Latin America, the Middle East and Africa [8]. Akeso keeps China and runs its own China-only trials. **This split matters for reading the data:** Akeso's trials are in Chinese patients, while Summit's are global and include Western patients.

Where things stand.

- **Akeso (China):** ivonescimab is approved in three indications:
 - second-line EGFR-mutated NSCLC (May 2024) [1]
 - first-line PD-L1-positive NSCLC as monotherapy (April 2025) [2]
 - first-line squamous NSCLC (August 2026) [16]
- **Summit (its own territories):** the first US filing covers second-line EGFR-mutated NSCLC. The FDA accepted it in January 2026 and has set a decision date (PDUFA) of **14 November 2026** [1].

Across both companies, 15 Phase III studies are underway, 4 of which are run by Summit globally [1].

Summit-sponsored global trial	Setting	Comparator	Status
HARMONi	2L EGFR-mutated NSCLC	Placebo + chemo	Basis of the US filing; PDUFA 14 Nov 2026 [1]
HARMONi-3	1L NSCLC, all PD-L1 levels; 600 squamous + 1,000 non-squamous patients	Pembrolizumab + chemo	Squamous final PFS 2H26; non-squamous 1H27 [4][15]
HARMONi-7	1L NSCLC, PD-L1 ≥50%	Pembrolizumab alone	Enrolling [1]

Summit-sponsored global trial	Setting	Comparator	Status
HARMONi-GI3	1L colorectal cancer	Bevacizumab + chemo	Enrolling [1]

Other programmes include:

- ILLUMINE, a head-and-neck cancer trial run by the cooperative group GORTEC [1].
- HARMONi-GU1, a Phase II/III trial in bladder cancer [16].
- Combination collaborations with Revolution Medicines and GSK [7].

Scorecard: what the new numbers say

Number	Read	Why it matters
HARMONi-2 OS HR 0.73 (95% CI 0.57–0.95), p=0.009 [2]	Good	First OS win vs Keytruda for any PD-(L)1 x VEGF bispecific. Removes the class's biggest objection.
Median OS 30.8 vs 22.6 months (+8.2 months); 398 patients, 234 deaths [2]	Good	A large absolute gain in first-line NSCLC, from a mature dataset.
PD-L1 ≥50% subgroup HR 0.58 (0.38–0.89) [2]	Good	Strongest signal. Maps directly onto Summit's HARMONi-7 (same mono-vs-mono design).
PD-L1 1–49% subgroup HR 0.85 (0.61–1.18) [2]	Weak	Interval crosses 1. HARMONi-3 enrolls all PD-L1 levels, so this group matters more than the headline.
Non-squamous subgroup HR 0.79 (0.55–1.14) [2]	Weak	Interval crosses 1. Non-squamous is HARMONi-3's larger cohort (1,000 of 1,600 patients) [15].

Number	Read	Why it matters
Squamous subgroup HR 0.65 (0.45–0.95) [2]	Mixed	Consistent with HARMONi-6 in China [19]. Yet the global HARMONi-3 squamous cohort missed its interim PFS [12].
Serious treatment-related AEs: 29.9% vs 21.6% [2]	Watch	Meaningfully more serious toxicity, partly from longer treatment (median 14 vs 10 cycles) [3]. A US prescriber will weigh this.
Discontinuations for AEs 4.1% vs 5.0% ; immune-related AEs 34.0% vs 33.7% [2]	Good	The extra toxicity isn't forcing patients off drug, and immune toxicity matches Keytruda.
HARMONi June-26 ITT OS HR 0.76 (0.61–0.95), nominal p=0.0151 [3]	Good	The interval now excludes 1. Submitted to the FDA ahead of the PDUFA date [1].
HARMONi Western subgroup HR 0.76 (0.52–1.10) [3]	Mixed	Point estimate matches Asia, but on 165 patients the interval still spans 1 [1].
Both later HARMONi cuts are ad hoc [1]	Bad	The trial missed OS at its primary analysis (HR 0.79, p=0.057) [1]. Later nominal p-values can't change that statistically.

1. HARMONi-2: what it changes

Until September, the fair criticism of the whole class was that no PD-(L)1 x VEGF bispecific had shown a survival benefit over Keytruda. HARMONi-2 ends that. Ivonescimab alone gave patients a median of 30.8 months against 22.6 months on pembrolizumab alone, and the result was statistically significant [2].

Four things follow:

- **The mechanism is validated against the right comparator.** This is no longer a PFS-only story.
- **HARMONi-7 looks materially de-risked.** It tests the same thing, ivonescimab monotherapy vs pembrolizumab monotherapy, in PD-L1 $\geq 50\%$ patients, where HARMONi-2 showed its strongest effect (HR 0.58) [2].
- **The whole class re-rates, not just Summit.** Large pharma already owns rival assets:
 - BMS paid \$3.5bn upfront to co-develop BioNTech's PD-L1 x VEGF bispecific [26].
 - Pfizer paid \$1.25bn upfront for 3SBio's candidate [25].
 - Merck paid \$588m upfront for LaNova's LM-299 [24].
 - AbbVie paid \$650m upfront for RemeGen's RC148 in January 2026 [23].

HARMONi-2 is evidence for all of them. Summit's lead is time, not exclusivity.

- **Partnering leverage improves.** Bloomberg reported in 2025 that AstraZeneca discussed a licence worth up to \$15bn [11]. No deal was announced, but an OS win against Keytruda strengthens Summit's hand in any future talks.

2. HARMONi-2: what it doesn't change

The result is from **China only**, from an **interim analysis**, on a **secondary endpoint**, using **monotherapy**, with data generated and analysed by Akeso [3]. Each qualifier matters for Summit specifically:

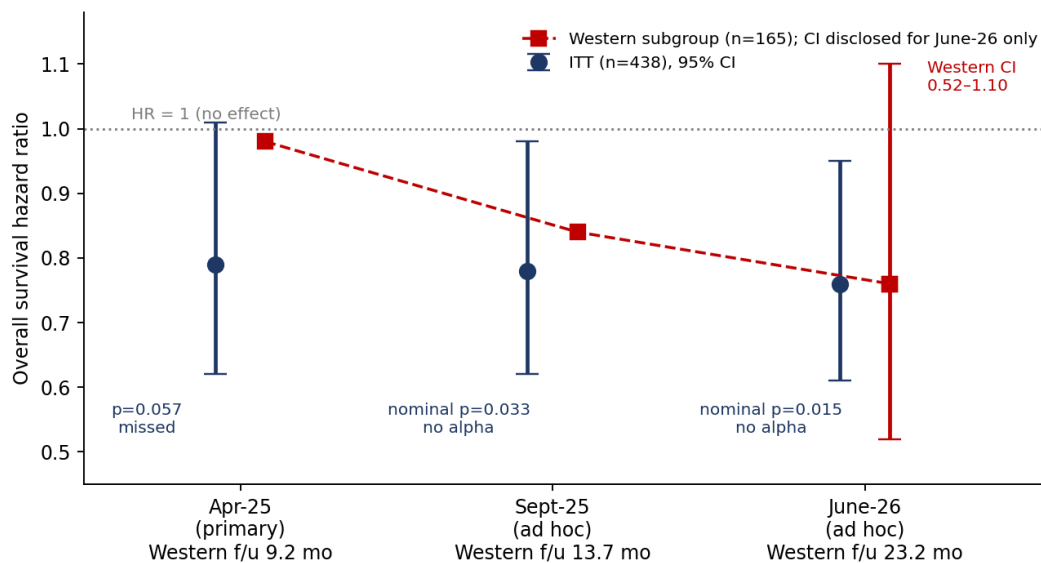
- **It can't carry a US filing on its own.** In 2022, the FDA's oncology advisory committee voted 14–1 that sintilimab needed additional trials applicable to US patients before a decision. Its application rested on a trial run only in China [31]. HARMONi-2 is supporting evidence for the FDA, not a basis for approval.
- **Monotherapy is not HARMONi-3.** HARMONi-3 adds chemotherapy to both arms [15]. Pembrolizumab plus chemotherapy is a much stronger control than pembrolizumab alone, so the room for a relative benefit shrinks.
- **The weaker subgroups are the ones HARMONi-3 depends on.** HARMONi-3 enrolls all PD-L1 levels, and non-squamous is its larger cohort [15]. In HARMONi-2, both the PD-L1 1–49% and non-squamous intervals cross 1 [2].
- **Every OS win so far is Chinese:**

- HARMONi-A [20]
- HARMONi-2 [2]
- HARMONi-6 [19]
- HARMONi-G11, in biliary cancer [17]

The only randomized Western data is HARMONi, where the survival result has improved but remains statistically unproven [1].

3. HARMONi: better, not finished

Exhibit 1 — HARMONi OS: the ITT interval now excludes 1, the Western interval does not



Source: Summit Therapeutics press releases (22 Jul 2026) and WCLC 2026 presentation. Asian patients locked at 32.7 mo median follow-up. Median OS 16.8 vs 14.0 mo is the ITT primary analysis (unchanged at Sept-25). Later cuts are ad hoc; no alpha was reserved.

Source: Summit press releases [1][3].

The Western hazard ratio has moved from 0.98 to 0.84 to 0.76 as follow-up lengthened from 9.2 to 23.2 months. It now matches the Asian result [1], and the full-trial interval excludes 1 [3]. That is genuinely positive for the 14 November FDA decision.

Two key caveats are noteworthy here. First, the Western interval (0.52–1.10) still includes no benefit [3]. Second, the later cuts were run after the primary analysis had missed [1], with no statistical allowance for re-testing. How the FDA weighs a clear PFS win, a narrowly missed OS endpoint, and supportive but unplanned updates is the real PDUFA question.

4. The readout that decides the stock

On 30 April, Summit disclosed that the **HARMONi-3 squamous cohort missed its interim PFS analysis** [12]. Shares opened down ~24% [13] from a prior close near \$21 [14].

There are two readings:

- **Benign.** The interim bar was set very high, and Summit said so. Citi estimated only ~0.001 alpha was spent on the interim [12].
- **Less benign.** The effect is smaller outside China and against pembrolizumab plus chemotherapy.

China's squamous data are strong. HARMONi-6 showed a ~40% PFS advantage over tislelizumab plus chemotherapy [12] and a significant OS benefit [19]. HARMONi-2's squamous subgroup had an OS HR of 0.65 [2]. That makes the global miss harder to dismiss, not easier.

Our base case: a statistically significant win at the final analysis, but with a smaller effect than in China. We expect a PFS HR nearer 0.75–0.80 than HARMONi-6's ~0.60. A benefit as large as China's would very likely have cleared even a strict interim bar.

Squamous final PFS	Read	Why
HR ≤ 0.70 , consistent in Western patients	Clearly positive	China effect translates on a chemo backbone. Supports a large first-line opportunity.
HR ~0.75–0.85 , significant	Mixed	A win, but a modest one. Commercial debate shifts to pricing against Keytruda, which loses US exclusivity in 2028 [22]. Our most likely outcome.
Miss , or benefit driven only by Asian sites	Clearly negative	Undercuts the translation thesis ahead of the non-squamous readout.

5. Balance sheet and financing

Number	Read	Comment
Cash + ST investments \$690.7m (30 Jun 2026), no debt [4][6]	Tight	The 10-Q states cash is not sufficient to fund planned operations for 12 months (going-concern language) [5].
Operating cash use ~\$140m in Q2 [6]; \$263.4m in 1H26 [5]	Rising	Roughly five quarters of cash at the current rate.
GAAP opex \$220.5m in Q2, up 13% on Q1 [6]; non-GAAP \$151.8m vs \$122.4m in Q1 [6]	Expected, but costly	Driven by HARMONi-3, HARMONi-7 and HARMONi-GI3 [6], with HARMONi-GU1 still to come [16].
ATM: \$230.8m raised in Q2, \$68.4m after quarter-end [4], plus a new \$380m facility [6]	Bad for holders	Summit is funding itself by selling stock into the market. Ongoing dilution is an overhang.
October 2025 PIPE: insiders \$272m at \$18.74 , Akeso \$10m [27]	Good	Management bought at roughly today's price.
Duggan stake ~75% [11]	Double-edged	A committed funder and stable control, but a small float amplifies moves, and any deal happens on his terms.

Financing risk is manageable but not free. Until approval or a partnership, the ATM is the funding source, and every rally doubles as a selling window.

6. Is the good news priced in?

Largely, yes. The stock has risen ~44% over the 30 days to ~20 September, to ~\$17.85 [28]. It sits a little below the ~\$21 it traded at before the April miss [14], so the market has mostly forgiven the interim result on the strength of the China data.

At ~\$14bn, investors are not paying for the second-line EGFR label, which is a small population. They are paying for a meaningful chance that ivonescimab takes a large share of **first-line lung cancer from Keytruda**, a franchise that sold \$31.7bn in FY2025 across all indications [21]. That outcome depends on HARMONi-3, which has not yet reported.

Sell-side views are split. Jefferies upgraded to Buy with a \$25 target ahead of the HARMONi-3 readout [29], while Barclays remains at Hold [30].

View: Neutral. The data are better than they were a month ago, but the stock has moved with them, and the ATM is supplying new shares into the strength.

- **We turn positive** on a squamous final PFS HR ≤ 0.70 with a Western subgroup pointing the same way.
- **We turn negative** on a squamous miss, a benefit confined to Asian sites, or a CRL citing lack of significant OS.

7. What to watch

Date	Event	Good would be	Bad would be
25 Oct 2026	HARMONi-GI1 (biliary) at ESMO Presidential Symposium [18]	Large OS gain vs durvalumab + chemo, clean safety: platform credibility beyond lung	Modest effect or new safety signal
14 Nov 2026	FDA decision, 2L EGFR-mutated NSCLC [1]	Approval with OS data in the label	CRL citing lack of statistically significant OS
2H 2026	HARMONi-3 squamous final PFS + interim OS [4]	HR ≤ 0.70 , consistent in Western patients	Miss, or a win driven only by Asian sites
1H 2027	HARMONi-3 non-squamous PFS [4]	Effect close to HARMONi-2's overall result	Effect near its weak non-squamous subgroup (HR ~ 0.79 , CI crossing 1)

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